GAMSA · Gender-Affirming Medicines South Australia

Blood test guide: Cyproterone acetate

Cyproterone acetate (androgen blocker) · gamsa.au/monitoring

Key points Liver tests before and during treatment. Tell your prescriber about new headaches or changes in your vision.

Applies to me	Test	When	Aim	Talk to your prescriber if
☐	**Liver function**	Before starting, regularly during treatment, and at once if you have signs of liver trouble	Within the lab's range	You have yellow skin or eyes, or dark urine: get checked straight away
☐	**Headaches and vision (meningioma watch)**	No routine scan. Your prescriber keeps your dose, and your total dose over the years, as low as works	The lowest dose that works	You have new or worsening headaches, changes in your vision, a seizure, or new weakness: tell your prescriber promptly
☐	**Total testosterone (and oestradiol)**	At each review; this is how cyproterone's effect is checked	Below 2 nmol/L, if you want full suppression	It stays above 2 nmol/L when you want full suppression
☐	**Full blood count, adrenal function and prolactin**	Full blood count before starting and regularly; adrenal function as the product information advises; prolactin if you have symptoms	Within the lab's range	You have new headaches, vision changes or milky nipple discharge (prolactin), or low mood

A guide to what to expect, not a schedule to follow on your own. Your prescriber sets the timing.

Reading your results

- Lab reports mark results as high or low using the sex on your Medicare or lab record. On hormones, that range may not fit your body. In the first year your prescriber may ask the lab for both the male and the female ranges.
- A single flagged result is a reason to talk with your prescriber, not to change a dose on your own.
- Units: Australian labs report oestradiol in pmol/L, testosterone in nmol/L, and haematocrit as a fraction (0.50 L/L is the same as 50%).

Things that can make a result look wrong

- Haemoglobin and haematocrit: read against the range for your affirmed gender once hormones have had time to work (about 3 months on testosterone, about 6 months on oestradiol). On testosterone, the male range is used.

Sources: AusPATH 2026 pp27, 30, 31, 42, 53, 54, Table 4; Androcur PI s4.3, s4.4; 2019 position statement lowest effective dose. Full references and detail for clinicians: gamsa.au/monitoring.

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